

Medication Initiation and Monitoring Timelines

Synthetic cardiology medicine checkpoints and review windows

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Retrieval rule

Retrieve when forming a monitoring or appointment plan. These fictional intervals apply only after a clinician has selected and authorized therapy.

1. Universal initiation timeline

When	Required action	Owner
Before order	Reconcile medicines/allergies; confirm indication, baseline observations, relevant labs and tests; document counselling needs	Prescriber + pharmacist
Day 0	Issue complete signed order and written directions; book required tests before patient leaves workflow	Prescriber + nurse/booking
48-72 hours	High-risk cases only: verify access, administration, early intolerance, and outstanding urgent tests	Nurse/pharmacist
7-14 days	Obtain drug-specific tests and observations when required; review results promptly	Diagnostics + results clinician
2-6 weeks	Assess symptoms, adherence, observations, adverse effects, and treatment goal	Cardiology team
3 months	Formal medication reconciliation and response review unless earlier review required	Clinician + pharmacist
Ongoing	Repeat at the fictional class-specific interval and after clinically significant changes	Named results owner

2. Simulated class monitoring matrix

Medicine group	Baseline	After start/change	Stable review	Key hold/escalation workflow
ACE inhibitor / ARB / ARNI	BP; renal function; potassium; indication-specific assessment	Renal function and potassium in 7-14 days; BP and symptoms within 2-4 weeks	At least every 3-6 months in this simulation	Do not auto-adjust. Urgently route marked potassium rise, acute kidney concern, symptomatic hypotension, pregnancy concern, or angioedema symptoms.
Mineralocorticoid receptor antagonist	BP; renal function; potassium	Potassium/renal function at approximately 3-7 days and again at 4 weeks	Every 1-3 months initially, then clinician-defined	No initiation or escalation without current results and named results owner.
Loop or thiazide-type diuretic	BP; volume assessment; renal function; sodium; potassium	Labs in 7-14 days; earlier when high risk; weight/symptom review	Every 3-6 months or as authorized	Escalate dehydration, confusion, weakness, syncope, major weight change, electrolyte flag, or renal deterioration.
Beta blocker	HR; BP; rhythm/ECG when relevant; congestion status	HR/BP and symptoms in 1-2 weeks after change	Every 3-6 months	Do not stop abruptly unless an authorized emergency instruction exists; escalate syncope, severe dizziness, marked bradycardia flag, or worsening congestion.
Calcium-channel blocker	BP; HR for rate-limiting agents; oedema status	BP/HR and symptoms within 2-4 weeks	Every 3-6 months	Escalate syncope, severe dizziness, marked bradycardia flag, or troublesome oedema.

Medicine group	Baseline	After start/change	Stable review	Key hold/escalation workflow
Anticoagulant	Indication; weight; renal/hepatic function; haemoglobin; bleeding risk; interacting drugs	Access/adherence and bleeding check within 1-2 weeks; lab timing per drug/order	At least every 3-6 months, more often if renal function changes	Urgent same-day review for significant bleeding, head injury, suspected clot, procedure conflict, dose uncertainty, or missed multiple doses.
Antiplatelet	Indication and intended duration; bleeding risk; interacting drugs	Tolerance and bleeding review in 2-4 weeks	At each cardiology/medication review	Never extend dual therapy duration by inference; verify stop/review date.
Statin / lipid-lowering	Lipid profile; liver assessment; interacting drugs; muscle symptoms	Lipids and liver tests at clinician-authorized interval, commonly 8-12 weeks in simulation	At least annually or per goal	Escalate severe unexplained muscle symptoms, dark urine, major liver-test flag, pregnancy concern, or interaction.
Antiarrhythmic	ECG/rhythm; renal/hepatic status; electrolytes; drug-specific organ baseline	Early ECG and drug-specific tests exactly as signed	Specialist-defined	Specialist initiation/oversight; agent must not infer monitoring from class alone.

3. Result ownership

- Every test order has a named individual or team responsible for reviewing the result.
- Normal results are acknowledged within two working days; abnormal or critical flags follow the diagnostic escalation policy.
- If the named reviewer is absent, the duty cardiology clinician assumes the queue.
- A test is not complete merely because a specimen was collected; completion requires result review, documented action, and patient communication where indicated.

4. Missed monitoring

If required safety monitoring is overdue, the system must not state that treatment is safe to continue. It should identify the overdue item, route it to the results owner, and provide the booking/escalation action specified in the signed plan.

- First failed contact: record attempt and use the approved alternate channel.
- Second failed contact for safety-critical monitoring: same-day clinician review of risk and supply plan.
- Do not threaten, blame, or automatically discharge a patient for non-attendance.

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